

Virtual Insurance Collection Employee

VICE

Some products already exist in market but fragmented: some products specialize in **AI voice agents that actually call insurance companies**.

Gateway EDI / TriZetto, Gateway supports claims, ERA/remittance, real-time eligibility, real-time claim status, authorizations/referrals and other payer transactions.

Some products worth looking

- [Infinx](#) — probably the closest to the **end-to-end AI RCM agent** you are describing. It has AI/automation agents for claim-status follow-up, prior authorization, eligibility, payment posting and denials, and says its platform connects through payer portals, APIs and clearinghouses. It specifically lists agents working with Availity, Optum, UHC and other payer systems.
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- [Operator Labs](#) — particularly interesting for the **telephone-call part**. Its AI agents call insurance companies, navigate IVRs, wait on hold, speak with payer representatives, obtain claim status/denial information, request reprocessing, document the conversation and automatically schedule follow-up calls.
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- [Waystar](#) — a much larger healthcare-payment platform moving toward an autonomous revenue cycle. Its 2026 AltitudeAI agentic platform addresses claims, denials, appeals, prior authorization and payment workflows. Waystar says its network covers more than one million providers and processes more than 7.5 billion transactions annually.

My plan can go one step further

We could build a “**virtual insurance collection employee**” for a physician practice or outpatient rehab center.

It could automatically:

Gateway/clearinghouse → AI Agent → Insurance company → Payment

The agent would pull every unpaid claim each morning and decide what to do:

Claim \$4,850 — 47 days outstanding

It checks Gateway electronically first.

Claim received
Status: pending
No payment issued

Instead of putting it into somebody's work queue, the AI takes over.

It calls the payer, navigates:

“For claim status, press 2...”

waits on hold, identifies itself as an automated representative of the provider where permitted, supplies the required provider/claim information, and asks:

“What is preventing payment on claim XXXXX?”

Suppose the insurance representative says:

“We need the operative report.”

The AI records that reason, obtains the reference/call number, retrieves the permitted document from the EHR/document system, prepares or submits it through the appropriate electronic workflow, and creates a follow-up:

Call payer again in 10 days if unpaid.

If the payer says:

“Claim was denied for timely filing.”

the agent can check the clearinghouse submission date. If the record shows timely filing, it can prepare the appeal and supporting proof for human approval—or automatically submit it where the organization's policies permit.

That is substantially more valuable than simply having an AI receptionist.

The product I would consider building

I'd make the central screen extremely simple:

\$1,284,620 Outstanding A/R

AI working: 327 claims

Payers called today: 86

Claims reprocessed: 21

Appeals submitted: 14

Payments recovered: \$37,450
Needs human attention: 9

The goal isn't merely "*AI calls insurance companies.*"

The goal is:

Give the AI the outstanding A/R and tell it: collect the money.

It uses electronic claim-status transactions first because they're cheaper and faster, payer portals/APIs second, and **telephone calls only when electronic channels cannot resolve the claim**. Humans receive only exceptions.

That could potentially replace a significant portion of repetitive insurance A/R follow-up work while keeping humans responsible for unusual cases, compliance-sensitive decisions, and negotiations.

And importantly, **you don't necessarily have to replace Gateway EDI**. Your AI layer could sit **above the existing clearinghouse** and integrate with the practice-management/EHR system, clearinghouse data, payer portals and telephone system. That makes the business much more practical.

ClearAuth AI

Imagine the doctor orders an **MRI of the lumbar spine**.

The AI agent automatically receives:

Patient + insurance + CPT 72148 + diagnosis + physician order.

Then it works through a decision tree:

1. Check electronically first

The agent determines:

Does this patient's insurance require authorization for CPT 72148?

It could use payer APIs, clearinghouse connections, Availity or payer portals.

If the answer is:

NO AUTHORIZATION REQUIRED

the AI records the result, date, source and reference/transaction number in the patient's record.

If PA is **required**, it proceeds automatically.

2. Try the online authorization route

The AI opens the appropriate payer/authorization portal, enters the patient and procedure information, uploads the order, office notes, imaging history and supporting clinical documentation, and submits the request.

This is already emerging. For example, [Akomi](#) says its AI can autonomously submit through payer portals or fax and track the authorization to resolution.

[Champ AI](#) describes an agent that checks payer requirements, assembles the clinical packet, submits through the required channel and continues following the authorization. It specifically describes using APIs and browser agents for payer portals.

And [Flexbone AI](#) describes both **voice and browser agents** that work across payer portals and phone lines.

3. If the portal doesn't work → AI calls the insurer

This is where voice AI becomes especially useful.

The AI could say:

"I'm calling on behalf of Dr. Smith regarding prior authorization for a patient."

It navigates the IVR, waits on hold, talks with the insurance representative, verifies requirements and obtains the authorization number/status.

Infinitus is already demonstrating this type of payer conversation.

4. The AI reports only exceptions to the staff

Instead of your employee spending **20–40 minutes chasing the insurance company**, their screen might simply show:

MRI L-Spine — APPROVED

Authorization: XXXXX

Valid: Aug 26–Sep 26

Reference #: XXXXX

Or:

⚠️ CLINICAL REVIEW REQUIRED

Missing: 6 weeks conservative treatment documentation

Human review needed

That last part is important. I would **not** design the system to make independent medical judgments. Let AI perform the repetitive administrative work and escalate medical-necessity questions, peer-to-peer reviews and unusual denials to qualified staff/clinicians.

There may be a business opportunity here

The interesting part is that I don't think you necessarily need to invent the underlying AI technology.

You could build a "**Prior Authorization AI Employee for Small Physician Practices**" by combining:

EHR → eligibility/clearinghouse → payer portal/browser agent → voice AI → documentation → EHR

The differentiator could be that the doctor doesn't care *how* the authorization gets obtained.

The doctor simply clicks:

GET AUTHORIZATION